



DIAGNOSTIC INTERVIEW AND HISTORY TAKING



**psych
scene**
RANZCP EXAM PREP



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THE DIAGNOSTIC INTERVIEW

PSYCHOSIS

The main disorders that one should consider when patient presents with psychotic symptoms are:

- Schizophrenia
- Bipolar Disorder
- Depression
- Drug Induced Psychosis
- Organic (epilepsy, head injury, vascular, immune mediated, infectious etc.)

Diagnostic interviewing will proceed along the above lines to establish the diagnosis.

DSM V CRITERIA FOR SCHIZOPHRENIA

Two or more of the following for a duration of one month; Criterion A; (1, 2 or 3 must be present)

1. Delusions
2. Hallucinations
3. Disorganized speech
4. Grossly disorganized or catatonic behaviour
5. Negative Symptoms
 - Continuous signs of the disturbance for 6 months
 - Schizophreniform disorder: episode lasts for at least one month but less than 6 months
 - Brief Psychotic Disorder: Episode lasts at least one day but less than one month
 - Delusional Disorder: One or more delusions with a duration of 1 month or longer and criterion A for schizophrenia has never been met (erotomantic, jealous, persecutory, somatic)

FIRST-RANK SYMPTOMS OF SCHIZOPHRENIA

- 1) **Delusions of Perception** – Primary delusion.
Delusion occurring out of the blue, e.g. ‘When I saw the red car passing by me yesterday, I knew I was going to be killed.’
- 2) **Auditory Hallucinations**
 - Third-person auditory hallucinations (arguing or talking about patient) – He or she
 - Running commentary on person’s actions or thoughts
 - Thought echo (*Gedankenlautwerden*) – Thoughts spoken aloud
- 3) **Delusions of Thought Interference**
 - Thought insertion (put into your head)

- Thought withdrawal (taken out of your head)
- Thought broadcasting (broadcast so that other people know what you are thinking)

4) **Passivity Phenomenon or Delusions of Control**

Control of the following by an external force:

- Impulses
- Actions
- Feelings
- Somatic Passivity – Passive recipient of bodily sensations imposed from outside forces. Cf. Somatic Hallucinations – somatic passivity is a delusion and somatic hallucination is a hallucination. It is sometimes difficult to differentiate the two.

DSM V CRITERIA FOR SCHIZOAFFECTIVE DISORDER

1. Major mood episode plus Criterion A of schizophrenia
2. Delusions or hallucinations for 2 or more weeks in the absence of a major mood episode
3. Criteria for mood disorder are fulfilled during active and residual periods of illness

First-rank symptoms are considered to be pathognomonic for schizophrenia in the absence of organic psychopathology. They can also occur in the following conditions:

- Mania: 15- 20% of patients have First Rank Symptoms
- Depression: 15% of patients have First Rank symptoms
- Amphetamine misuse
- Epilepsy (especially temporal lobe epilepsy)

DIAGNOSTIC INTERVIEW FOR SCHIZOPHRENIA

OPEN QUESTIONS

Tell me what symptoms of schizophrenia you have? (If patient tells you diagnosis. It also gives an understanding of insight.)

It is not unusual/Sometimes when people are stressed, they can have strange experiences. Have you ever experienced this?

Can you give me some examples?

CLOSED QUESTIONS DIRECTED TOWARDS ESTABLISHING A DIAGNOSIS

- *Have you ever heard voices/people talking about you when no-one else is around/you are on your own? (Auditory hallucinations)*
- *Where do they come from – from inside your head or through your ears?*
- *How often do you hear them?*
- *How do they affect you?*
- *If I were to hear them what would I hear?*
- *Can you tell me exactly what they say?*
- *Do they ever comment on your actions?*
- *Do they ever tell you to harm yourself or others? (Important: Risk, Command hallucinations)*
- *Have you ever felt/Do you feel like your thoughts are being interfered with..... pause.....like thoughts being put into your head (Thought insertion), thoughts being taken out of your head (Thought withdrawal) or even broadcasted to other people so that they know what you are thinking (Thought broadcasting)*
- *Have you ever felt under the control of someone else other than yourself?*
- *Have you ever felt like your actions, feelings or urges are being controlled by some external force? (Passivity phenomenon)*
- *Have you felt any strange sensations in your body? (Somatic passivity)*
- *Have you felt that people on the radio or TV were talking about you in their reports, or giving you special messages?*
- *Do you ever receive any special messages from books, radio or TV that are specifically meant for you? (Paranoid bizarre delusions)*
- *Do you ever feel like you are being spied upon, like someone is following you or conspiring against you?*
- *Do you ever feel like you are being watched by cameras or special devices?*
- *How did it start? (To establish delusional perception)*
- *Have you had any strange tastes, smells, lately?*
- *Have you felt strange things in your body that other people couldn't?*



IMPORTANT POINT

Olfactory hallucinations should be excluded as temporal lobe epilepsy can mimic schizophrenia.

Visual, tactile and gustatory(taste) hallucinations can signal an organic cause.

COMORBIDITY IN PSYCHOSIS

Drug misuse/dependence – cannabis, amphetamines
Bipolar Disorder – ask about manic symptoms
Depression – assess Mood
Obsessive Compulsive Disorder • high rates in schizophrenia • exacerbated by most atypical antipsychotics, especially Clozapine
Social phobia – high rates in schizophrenia • especially common in chronic, long-stay ward patients
Agoraphobia and additional anxiety symptoms • common in schizophrenia
If relevant ask for schizotypal personality/schizoid personality traits in early adolescence/adulthood

DEPRESSION (LOW MOOD)

The main disorders that one should keep in mind when patient presents with low mood are:

- Depression
- Bipolar Disorder
- Personality Disorder – mainly Borderline Personality Disorder
- Anxiety Disorders, Drug induced Mood Disorder
- Organic Disorders – These can cause low mood due to direct and indirect effects (psychological adjustments, medications)

DSM-V CRITERIA FOR DEPRESSION

Five or more of the following symptoms for two weeks (At least one of the symptoms must be depressed mood or loss of interest or pleasure)

- | |
|--|
| <ul style="list-style-type: none"> • Depressed mood • Loss of interest or pleasure • Significant appetite or weight loss or gain • Insomnia or hypersomnia • Psychomotor agitation or retardation • Fatigue or loss of energy • Feelings of worthlessness or excessive guilt • Impaired thinking or concentration; indecisiveness • Suicidal thoughts/thoughts of death |
|--|

Psychotic symptoms in depression indicate psychotic depression

- Delusions of guilt
- Hypochondriacal delusions
- Delusions of nihilism
- Delusions of poverty
- First-rank symptoms may occur in 15% of cases

DIAGNOSTIC INTERVIEWING FOR DEPRESSION

OPEN QUESTION – *How have you been feeling in yourself lately?*

CLOSED QUESTIONS

Mood

- *Have you felt low or depressed in yourself lately?*
- *Have you felt low/depressed/tearful every day, for most of the day, in the last two weeks?*
- *How does that affect you? (empathy) It must be difficult to feel like that over such a long period. Tell me a bit more about the way you feel.*
- *Does your mood improve or get worse as the day goes on (Diurnal variation)?*
- *When was the last time you smiled? (Mood)*
- *On a scale of 1 to 10, if 10 was really, really down, where would you put yourself?*

Anhedonia

- *What are kinds of things that you normally enjoy doing? (Anhedonia)*
- *Have you been enjoying these activities as much as you've done in the past? (Anhedonia)*
- *When was the last time you were involved in (activity) (Anhedonia)?*

Anergia

- *If have children/or working – Having two children must be quite a handful, I wonder how your energy levels keep up? (Anergia)*
- *Have you been feeling unusually tired in yourself lately? (Anergia)*
- *How have your energy levels been over the past couple of weeks?*

Concentration/Decision making

- *Have you been able to focus on things lately?*
- *What's your concentration like?*
- *When you watch TV, are you able to follow programmes?*
- *Do you find it difficult to read books because you can't concentrate? (if mention of reading as a hobby)*
- *Have you had problems making decisions?*
- *How has that affected you? (empathy)*
- *Can you watch a half-hour television show from start to finish without losing your focus?*

Appetite

- *Since you've been feeling low, have you noticed any changes in your appetite?*
- *Since you've been feeling low, have you noticed any changes to your weight?*

- *Has it increased, decreased or stayed the same?*
- *Have your clothes gotten any bigger or smaller on you?*
- *Do you actually enjoy food as you usually would?*

Sleep

- *What has your sleep been like lately?*
- *Do you find it difficult to fall asleep? (Initial insomnia)*
- *Do you find it difficult to stay asleep? (Middle insomnia)*
- *What time do you wake up? (Early morning wakening)*
- *Is that usual for you?*

Hopelessness, guilt, worthlessness

- *How do you see the future?*
- *Do you see a future?*
- *Is there anything hurting your conscience?*
- *Do you ever blame yourself for what you are going through/experiencing?*
- *How do you feel about yourself as a person?*
- *How would you describe your confidence, self-esteem?*
- *Do you feel other people would be better off without you?*

Suicide (see later)

- *Have things got so bad, that you feel like life is not worth living?*
- *What thoughts have you had?*
- *How close have you come to doing any of the things you mentioned?*
- *What stops you?*
- *It is not unusual/sometimes when people feel low, they may have thoughts of suicide/not wanting to be here... Have you ever felt that way?*

CO- MORBIDITIES IN DEPRESSION

Bipolar Disorder – always ask about mania

Borderline Personality Disorder – duration of low mood is not sustained, and mood instability is prominent. Both may co-exist

Adjustment Disorder with Depressed Mood

- Mild features of depression in context of identifiable stressor, eg grief

Anxiety

- Very common co-morbidity in depression

Drug Induced Mood Disorder

- Alcohol Induced Mood Disorder is common
- Cannabis is associated with “amotivational syndrome”

Obsessive Compulsive Disorder

- OCD can lead to secondary depression

- Depression can exacerbate OCD

Organic

- Hypothyroidism
- Multiple Sclerosis



IMPORTANT

If patient has had experiences of low mood all their life, then consider the possibility of Borderline Personality Disorder or Dysthymia
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If DSM criteria for depression established, then diagnosis is Dysthymia



In older (> 55) patients depression presents atypically.

- Psychomotor retardation
- Pseudodementia
- Hypochondriacal overvalued/delusions
- Obsessive compulsive symptoms
- Agitated depression
- Apathy

Vascular Depression or early sub-cortical dementia is a hypothesis in elderly individuals.

DIFFERENTIATION BETWEEN GRIEF AND DEPRESSIVE DISORDER

The depressive disorder is characterised by:

- Guilt about things other than actions taken or not taken by the survivor at time of death
- Thoughts of death other than the survivor feeling that he or she would be better off dead or should have died with the deceased
- Marked psychomotor retardation
- Hallucinatory experiences other than thinking that he or she hears the voice of or transiently sees image of dead person
- Delayed reaction – mummification or feeling of being stuck
- Prolonged and marked functional impairment
 - **According to DSM V**
- In grief the predominant affect is feelings of emptiness and loss, while in MDE, it is persistent depressed mood and the inability to anticipate happiness or pleasure
- The dysphoria in grief is likely to decrease in intensity over days and weeks and occurs in waves (Pangs of grief) which are associated with thoughts or reminders of the deceased. The depressed mood of MDE is more persistent and not tied to specific thoughts and preoccupations

- The pain of grief may be accompanied by positive emotions and humour as opposed to pervasive unhappiness and misery in MDE
- The thought content in grief is generally of preoccupation with thoughts and memories of the deceased, rather than the self-critical pessimistic ruminations in MDE
- In grief self-esteem is preserved, whereas in MDE, self-loathing and feelings of worthlessness are common. Self-derogatory thoughts in grief if present is related to perceived failings related to the deceased
- If a bereaved individual thinks about death and dying, such thoughts are generally focussed on the deceased and possibility of “joining” the deceased whereas in MDE, such thoughts are focused on ending one’s own life because of feeling worthless, undeserving of life, or unable to cope with the pain of depression

BIPOLAR DISORDER

If patient presents with mood instability, consider the following possible diagnoses:

- Bipolar I
- Bipolar II
- Schizoaffective disorder
- Cyclothymia
- Borderline Personality Disorder

DSM-IV CRITERIA FOR MANIC EPISODE (BIPOLAR DISORDER)

1. A DISTINCT PERIOD OF ABNORMALLY AND PERSISTENTLY ELEVATED, EXPANSIVE OR IRRITABLE MOOD, LASTING AT LEAST ONE WEEK (OR ANY DURATION IF HOSPITALIZATION IS NECESSARY)
2. DURING THE PERIOD OF MOOD DISTURBANCE, THREE OR MORE OF THE FOLLOWING
 - Inflated self-esteem or grandiosity including grandiose delusions
 - Decreased need for sleep
 - More talkative than usual
 - Flight of ideas or subjective experience that thoughts are racing
 - Distractibility
 - Increase in goal directed activity or psychomotor agitation
 - Excessive involvement in pleasurable activities that have a high potential for painful consequences (e.g. sexual indiscretions, business investments)

Δ Hypomanic episode has all the above criteria but is not severe enough to cause marked impairment in social or occupational functioning.

Bipolar I	At least one episode of mania (one or more episodes), previous episodes may have been depression or hypomania
Bipolar II	At least one hypomanic episode
Cyclothymia	Presence of numerous hypomanic episodes intermingled with depressive episodes that do not meet full criteria for major depressive episodes
Rapid Cycling Disorder	Four or more episodes of major depression, manic, hypomanic or mixed episodes occurring during a twelve-month period
Mixed Episode	Depression and mania occur simultaneously

DIAGNOSTIC INTERVIEW FOR MANIA (BIPOLAR DISORDER)

OPEN QUESTIONS

- *Have you ever had the opposite of depression, when you've been extremely happy, over the top, doing things out of character, talking too fast? (Elevated mood)*
- *How long did it last for? (cf Borderline construct)*

CLOSED QUESTIONS

Grandiosity

- *During this period did/do you feel especially confident, like you were on top of the world?*
- *How do/did you see yourself in comparison to others?*
- *Did/do you feel you had special powers that others didn't?*
- *Did/do you feel a special connection with God?*
- *Do you consider yourself special in any way?*
- *Do you think God has a special purpose for you here?*

Flight of Ideas

- *Do you find that your thoughts are racing?*

Pressure of Speech

- *Do you find it hard to stop talking?*
- *Have your friends/family commented on the way you are talking?*
- *Have they said that they find it difficult to follow your speech?*

Sleep Deficit

- *Have you needed much less sleep than usual?*
- *Do you feel like you can work all day without needing rest/sleep?*

Increased Goal Directed Activity

- *Have you taken on any new activities lately?*
- *Have you come across any brilliant ideas lately?*

Indiscretion/Recklessness

- *Have you been in trouble with the police or your family lately?*
- *Have you been doing things that are out of character for you?*

Differentials and Co-morbidities

- Schizoaffective Disorder – Criteria of Schizophrenia should be present
- Personality Disorder – esp. Borderline Personality Disorder. Both can co-exist
- Substance misuse – High degree of alcohol and drug misuse in Bipolar patients

ADHD

- Common in childhood
- Highly co-morbid

Drug induced /ECT induced mania

- Venlafaxine and TCAs have a higher likelihood of switch

SUICIDAL IDEATION

- Suicidal ideation can occur in any psychiatric disorder
- Suicide and self-harm have different clinical correlates and risk factors
- Bipolar Disorder, Depression, Schizophrenia have rates of 20–30%; 15–20% and 10% respectively for suicide

RISK FACTORS FOR SUICIDE INCLUDE

- Male
- Elderly
- Previous suicide attempt
- Depression
- Social isolation, e.g. widowed
- Alcohol and drug misuse
- Physical illness

QUESTIONS TO ESTABLISH SUICIDE RISK AND TO EVALUATE PATIENT'S INTENT AND PLAN

- *Sometimes when people feel low, they may find life not worth living. Have you felt that way?*
- *What thoughts of suicide have you had? (Ideation)*
- *How long have you been thinking about this? (intent/severity)*
- *What have you planned?*
- *Have you thought of cutting yourself?*
- *Have you thought of taking an overdose?*
- *Have you thought of hanging yourself?*
- *Have you thought of jumping off a bridge?*

- *Have you thought of shooting yourself?*
- *Do you have any weapons at home?*
- *Do you have any pills/rope at home?*
- *How close have you come to doing any of this/hurting yourself?*
- *Have you taken pills out of the bottle on your hand/tied a noose/gone and stood at a bridge? (plan)*
- *What stopped you? (protection factors)*
- *Is there anything to live for?*
- *Have you written a suicide note? (intent)*
- *Have you made arrangements to put your affairs in order after your death, like made a will? (intent)*

If patient has self-harmed or attempted suicide and assessment is after that, then

- Go through attempt in detail
- *How do you feel now that you have survived?*
- *Are you still feeling suicidal? Do you have any specific plans?*

In clarifying the attempt, ask specifically about intoxication and impulsivity during attempt (present in Personality Disorder)

ANXIETY

The following are the differential diagnoses that may apply to patients presenting with anxiety

- Generalised Anxiety Disorder (GAD)
- Social Phobia
- Agoraphobia and Panic Disorder
- Anxiety and Depression
- Obsessive Compulsive Disorder
- Post-Traumatic Stress Disorder

GENERALISED ANXIETY DISORDER

DSM-V CRITERIA FOR GAD

- Excessive anxiety and worry, occurring more days than not for at least six months, about a number of activities
- Difficulty controlling the worry
- It is associated with three or more of the following:
 - Restlessness
 - Being easily fatigued
 - Difficulty concentrating or mind going blank
 - Irritability
 - Muscle tension
 - Sleep disturbance

1. Marked fear or anxiety about two (or more) of the following five situations: using public transportation, being in open spaces, being in enclosed spaces, standing in line or being in a crowd and being outside the home.
2. The individual fears or avoids these situations because of thoughts that escape might be difficult or help might not be available in the event of developing panic like symptoms or other incapacitating or embarrassing symptoms.
3. The situations almost always provoke fear or anxiety.
4. The agoraphobic situations are actively avoided, require presence of a companion, or are endured with intense fear or anxiety.
5. The fear or anxiety is out of proportion to the actual danger posed by the agoraphobic situations and sociocultural context.
6. Duration is 6 months or more.

Panic Disorder

1. Recurrent unexpected panic attacks
2. At least one of the attacks has been followed by one month or more of the following:
 - Persistent concern about having further attacks
 - Significant change in behaviour related to attacks

Social Phobia

- A marked and persistent fear of one or more social or performance situations in which a person is exposed to possible scrutiny by others. The individual is worried that he/she will act in a way that will be negatively evaluated (e.g. be embarrassing or humiliating).
- Avoidance of the situation or endured with intense fear
- Fear or anxiety is out of proportion to the actual threat posed by social situation
- Duration 6 months

DIAGNOSTIC INTERVIEW FOR GAD

OPEN QUESTIONS

- *Are you a worrier?*
- *What do you worry about?*
- *Have you been worrying about simple things you shouldn't be worrying about?*
- *Over the past few months of worrying, have you noticed that you have been jittery, on edge? Irritable?*
- *Headaches, tension in your head and neck?*
- *What has your sleep been like? (Initial Insomnia is common)*
- *Do you have problems concentrating?*

DIAGNOSTIC INTERVIEW FOR PANIC DISORDER

- *Have you ever had a panic or anxiety attack?*
- *Can you describe it to me?*
- *What I mean is, a sudden rush of fear and anxiety in which your heart beats really fast, you get short of breath, have tingling around your mouth and fingers, butterflies*

in your stomach, feel like you are going mad or going to die. Has that ever happened to you?

- *When you have a panic attack does it come out of the blue or does it happen in specific situations? (for Panic Disorder, they happen out of the blue)*
- *How do they affect you?*
- *How often do you have them?*

DIAGNOSTIC INTERVIEW FOR AGORAPHOBIA

- *Does your anxiety ever interfere with shopping, driving, travelling on public transport?*
- *Have you started avoiding things because of your panic attacks?*
- *Ask specifically which ones.*
- *Do you have problems in travelling on the bus, train, crowded places like shopping centres?*
- *How do you cope with it?*
- *Who does the shopping?*
- *How do you take your children to school?*

DIAGNOSTIC INTERVIEW FOR SOCIAL PHOBIA

- *Does your anxiety interfere with you attending social situations?*
- *How uncomfortable do you get? Do you have a panic attack?*
- *As a result of this would you/have you avoided social situations?*



IMPORTANT POINTS

- A general screening question should be asked in most cases as anxiety is a highly co-morbid disorder that can affect social and occupational functioning.
- High levels of anxiety can be associated with drug and alcohol misuse.
- Social Phobia is associated with high levels of alcohol dependence

OBSESSIVE COMPULSIVE DISORDER

DSM-V CRITERIA FOR OCD

A. Presence of obsessions, compulsions or both

Obsessions are

- Recurrent and persistent thoughts, impulses or images that are intrusive and cause marked anxiety or distress
- These are usually met with attempts to ignore, or suppression or neutralisation with some other thought or action

Compulsions are

- Repetitive behaviours (e.g. washing, ordering, checking, hoarding) or mental acts (e.g. praying, counting, repeating words silently)
- The person feels compelled to perform in response to thought
- Behaviours are aimed at preventing or reducing distress

B. Obsessions or compulsions are time consuming

C. Impact on social and occupational functioning

DIAGNOSTIC INTERVIEW FOR OCD

OPEN QUESTIONS

- *Are you the sort of person that needs to do things in a particular order and if you don't you get distressed about it?*
- *What about constantly checking things, feeling dirty or having annoying thoughts pop into your mind over and over?*
- *Are you an obsessive person?*

CLOSED QUESTIONS

Intrusive Nature

- *Do you ever have thoughts or impulses that come into your mind even though you try not to have them?*
- *Do you find them intrusive?*
- *How do they make you feel in yourself? (anxiety-producing thoughts)*
- *Where do they come from? Are they your own thoughts? (c.f. Thought insertion in schizophrenia where the thoughts come from an outside source) (This is no longer necessary in the DSM-V)*
- *Resistance: How hard is it to resist the thoughts or actions?*
- *Are there any situations that you avoid related to these thoughts or actions?*

Compulsions

- *What do you do to relieve the anxiety?*
- *Does it make you feel less anxious?*

- *What would happen if you didn't perform the action?*
- *Do you have any other ways of neutralising the annoying thoughts?*
- *Some people may pray or repeat words or count. Do you do anything like that?*
- *Do you think these thoughts are senseless/excessive or unreasonable?*
- *How do they affect you?*
- *Does it affect your work?*



IMPORTANT POINTS

- There may be specific domains of symptoms, e.g. aggressive thoughts, sexual/religious, symmetry/counting, hoarding collecting, somatic.
- Be aware of **Schizo-Obsessive Disorder**, in which obsessive thoughts are on a spectrum which may encroach on to the psychotic spectrum. Here, thoughts are bizarre and may have the characteristic of an overvalued ideation.
- OCD may be associated with Tourette's disorder, trichillotomania and stereotypic movement disorder, e.g. skin picking, scratching, etc.

POST-TRAUMATIC STRESS DISORDER

DSM-V CRITERIA FOR POST-TRAUMATIC STRESS DISORDER (PTSD)

CRITERION A: STRESSOR

The person was exposed to: death, threatened death, actual or threatened serious injury, or actual or threatened sexual violence, as follows: (one required)

1. Direct exposure.
2. Witnessing, in person.
3. Indirectly, by learning that a close relative or close friend was exposed to trauma. If the event involved actual or threatened death, it must have been violent or accidental.
4. Repeated or extreme indirect exposure to aversive details of the event(s), usually in the course of professional duties (e.g., first responders, collecting body parts; professionals repeatedly exposed to details of child abuse). This does not include indirect non-professional exposure through electronic media, television, movies, or pictures.

CRITERION B: INTRUSION SYMPTOMS

The traumatic event is persistently re-experienced in the following way(s): (one required)

1. Recurrent, involuntary, and intrusive memories. Note: Children older than six may express this symptom in repetitive play.
2. Traumatic nightmares. Note: Children may have frightening dreams without content related to the trauma(s).
3. Dissociative reactions (e.g., flashbacks) which may occur on a continuum from brief episodes to complete loss of consciousness. Note: Children may reenact the event in play.
4. Intense or prolonged distress after exposure to traumatic reminders.
5. Marked physiologic reactivity after exposure to trauma-related stimuli.

CRITERION C: AVOIDANCE

Persistent effortful avoidance of distressing trauma-related stimuli after the event: (one required)

1. Trauma-related thoughts or feelings.
2. Trauma-related external reminders (e.g., people, places, conversations, activities, objects, or situations).

CRITERION D: NEGATIVE ALTERATIONS IN COGNITIONS AND MOOD

Negative alterations in cognitions and mood that began or worsened after the traumatic event: (two required)

1. Inability to recall key features of the traumatic event (usually dissociative amnesia; not due to head injury, alcohol, or drugs).
2. Persistent (and often distorted) negative beliefs and expectations about oneself or the world (e.g., "I am bad," "The world is completely dangerous").
3. Persistent distorted blame of self or others for causing the traumatic event or for resulting consequences.
4. Persistent negative trauma-related emotions (e.g., fear, horror, anger, guilt, or shame).
5. Markedly diminished interest in (pre-traumatic) significant activities.
6. Feeling alienated from others (e.g., detachment or estrangement).
7. Constricted affect: persistent inability to experience positive emotions.

CRITERION E: ALTERATIONS IN AROUSAL AND REACTIVITY

Trauma-related alterations in arousal and reactivity that began or worsened after the traumatic event: (two required)

1. Irritable or aggressive behaviour

2. Self-destructive or reckless behaviour
3. Hypervigilance
4. Exaggerated startle response
5. Problems in concentration
6. Sleep disturbance

CRITERION F: DURATION

Persistence of symptoms (in Criteria B, C, D, and E) for more than one month.

Note specifiers: delayed expression (6 months) and with dissociative symptoms

DIAGNOSTIC INTERVIEW FOR PTSD

OPEN QUESTIONS

- *What is the most traumatic /frightening experience you have had in your life?*
- *How does it affect you now?*

CLOSED QUESTIONS

Traumatic Event

- *Could you describe the event?* (ask first if the person has difficulty talking about the event)
- *How often do you think about it?*
- *Do you ever have thoughts about the event that seem to force their way into your mind?*
- *Do you ever have nightmares about the event?*
- *Do you ever get flashbacks?*
- *Do you ever feel like it's happening all over again?*
- *Do you have difficulties remembering the accident?*

Avoidance

- *Do you find yourself avoiding things that you associate with the memory?*
- *Have you been back to the place where the event took place?*
- *How hard is it for you to talk about the event?*
- *Are there any activities that you avoid?*

Numbing (Alterations in Cognitions and Mood)

- *How has trauma affected your life?*
- *Has it affected the way you feel?*
- *Have friendships suffered?*
- *Are you still able to have feelings for loved ones?*
- *Do you ever feel numb?*
- *How do you see the future?*

- *Has the trauma affected your interest in activities?*
- *Do you blame yourself/ Have you been feeling guilty about the event?*

Arousal

- *Since the trauma, have you felt on edge much of the time?*
- *Do you startle easily?*
- *Have you been feeling irritable?*
- *What is your sleep like?*

Ask about sleep, concentration.



IMPORTANT POINTS

- PTSD is highly associated with alcohol misuse/dependence
- Co-morbid conditions include:
 - Anxiety spectrum disorders
 - Depression
- High risk of suicide

COMPLEX POST-TRAUMATIC STRESS DISORDER

Complex PTSD occurs due to chronic, repeated trauma that is normally associated with the following, leading to chronic victimization

- Child sexual abuse
- Long-term, severe physical abuse
- Long-term domestic violence
- Concentration camps
- Chronic abuse in brothels

CLINICAL FEATURES

Individual has experienced a prolonged period (months to years) of total control by another

- Alterations in emotional regulation
- Alteration in consciousness
- Changes in self perception
- Alteration in perception of perpetrator
- Alteration in relations with others
- Changes in one's system of meaning

The condition has several features in common with Borderline Personality Disorder

ALCOHOL DEPENDENCE

DSM-V CRITERIA FOR ALCOHOL DEPENDENCE

2 of the following criteria leading to significant impairment and distress:

1. Alcohol is often taken in larger amounts or over a longer period than was intended
2. There is a persistent desire or unsuccessful efforts to cut down or control alcohol use
3. A great deal of time is spent in activities necessary to obtain alcohol, use alcohol, or recover from its effects
4. Craving, or a strong desire or urge to use alcohol
5. Recurrent alcohol use resulting in a failure to fulfil major role obligations at work, school or home
6. Continued alcohol use despite having persistent or recurrent social or interpersonal problems caused or exacerbated by the effects of alcohol
7. Important social, occupational, or recreational activities are given up or reduced because of alcohol use
8. Recurrent alcohol use in situations in which it is physically hazardous
9. Alcohol use is continued despite knowledge of having a persistent or recurrent physical or psychological problem that is likely to have been caused or exacerbated by alcohol
10. Tolerance
11. Withdrawal

CLOSED QUESTIONS / SCREENING QUESTIONS

- *Do you ever cope with your difficulties by having a drink/drinking/using drugs?*
- *What is your drug of choice?*
- *How often do you drink?*
- *When did you first start drinking?*
- *When were you drinking most heavily?*
- *How much have you been drinking over the last (period)?*

Difficulty controlling

- *Do you feel you can control your drinking?*
- *Can you stop at one drink?*
- *Do you crave for a drink (compulsion; Edward & Gross criteria)*

Tolerance

- *Have you found that your intake has gone up over the years?*
- *Have you found that you've needed more drinks to get the same high/effect?*

Withdrawal

- *Do you ever get the shakes when you stop drinking?*
- *Have you ever had shaken or fits when you stop drinking?*

Neglect of alternative pursuits

- *Do you find that drinking has become the most important thing in your life?*
- *What role does alcohol play in your life?*
- *Do you neglect your hobbies or family for drink?*

Use despite psychological and physical harm

- *How has alcohol affected your relationships? Your work?*
- *Have you got in trouble with the police because of your drinking?*

Motivation

- *Why do you drink?*
- *What are the good things about drinking for you?*
- *What are the less good things about drinking?*
- *What's been your longest period without a drink?*

ALCOHOL MISUSE

A maladaptive pattern of alcohol use leading to

- Failure to fulfil major role obligations at work, school or home
- Recurrent alcohol use in situations in which it is physically hazardous
- Recurrent alcohol-related problems with law-breaking
- Continued alcohol use despite persistent problems



IMPORTANT POINTS

- It is important to establish the difference between dependence and misuse
- Attempt to establish the temporal relationship of alcohol to psychiatric disorder

OPIATE USE

The same questions are asked to establish opiate dependence or misuse.

ADDITIONAL QUESTIONS TO ASK IN CASES OF OPIATE USE

- *What did you start with?*
- *When did you start using it?*
- *What did you try next?*
- *How often do you use heroin?*
- *How do you use it (snorting, injecting, chasing)?*
- *How much heroin would you use on a typical day? (ask in monetary value, amount)*
- *Over the last month, how often have you used it?*
- *On average, how much do you spend in a week?*
- *Are you aware of the problems associated with injecting?*
- *Do you share needles?*
- *Have you had tests for HIV and Hepatitis?*
- *Have you ever had complications from injecting?*

ANOREXIA NERVOSA

DSM-V CRITERIA FOR ANOREXIA NERVOSA

1. Restriction of energy intake relative to requirements, leading to a significantly low body weight in the context of age, sex, developmental trajectory, and physical health.
2. Intense fear of gaining weight or becoming fat or persistent behaviour that interferes with weight gain, even though at significantly low weight
3. Body image disturbance – disturbance in the way one's body weight or shape is experienced.

Restricting type or Binge-Eating/Purging type

Mild: BMI ≥ 17 kg/m²

Moderate: BMI 16-16.99 kg/m²

Severe: BMI 15-15.99 kg/m²

Extreme: BMI < 15 kg/m²

BULIMIA NERVOSA

DSM-V CRITERIA FOR BULIMIA NERVOSA

1. Eating in a discrete period of time (e.g within a 2 hour period), an amount that is definitely larger than what most individuals would eat in a similar period of time under similar circumstances.
2. A sense of lack of control over eating during the episode.
3. Compensatory behaviour to prevent weight gain, e.g. self induced vomiting, laxatives, enemas, diuretics.
4. The above behaviours occur at least once a week for three months.
5. Self-evaluation is unduly influenced by body shape and weight.

DIAGNOSTIC INTERVIEW FOR ANOREXIA NERVOSA

OPEN QUESTIONS

Patients are usually resistive and establishing rapport is crucial. They may minimise reported symptoms.

- *Take me through a normal day.*
- *What do you eat at breakfast, lunch, dinner?*
- *What do you think about your meal intake?*
- *How much do you weigh? How tall are you? (BMI)*

Morbid fear of fatness

- *Are you concerned about your weight?*
- *What do you do to prevent putting on weight?*
- *Are you afraid of gaining weight?*

Body image disturbance

- *What do you see when you look in the mirror?*
- *What is your ideal weight?*
- *Here is a piece of paper. Can you draw an outline of what you look like now and what you would like to look like?*

Self-induced weight loss

- *How do you lose weight/prevent putting weight on? Sometimes people may use other ways of losing weight such as exercise, thyroid supplements, fat burners, laxatives*

Complications

- *Sometimes people, who lose weight rapidly or have below normal body weight may have medical problems.*
- *Have you had any medical problems?*
- *Have you recently been admitted to hospital ...? For fractures, fainting?*
- *How have your menstrual periods been lately?*

DIAGNOSTIC INTERVIEW FOR BULIMIA NERVOSA

- *How often do you think of food? Tell me your thoughts.*
- *Do you spend a long time thinking about food/mealtimes/cooking, etc?*
- *Are there any particular rituals you have while eating or preparing food?*
- *Do you ever have bingeing periods? What I mean is eating large quantities of food in a short span of time, more than most people would eat.*
- *Do you feel out of control when you binge?*
- *How do you feel after eating?*
- *What do you do next?*
- *Do you induce vomiting or take laxatives after bingeing?*



IMPORTANT POINTS

- Physical examination is a must
- Brief cognitive examination, e.g. clock-drawing test with frontal lobe testing
- High degree of co-morbidity especially in bulimia nervosa. Rule out anxiety, depression, and substance misuse.
- Personality Disorder of Borderline type must be excluded as both conditions are closely associated with it.
- Bulimia is associated with shop lifting.
- Obsessional personality traits may also be present
- Family history of overweight/obesity or eating disorder.
- Personal history may reveal role models, childhood sexual abuse, attachment difficulties, bullying, and obesity as a child.
- History will reveal worsening of eating disorder with stressors.

HISTORY TAKING CONTINUED

This chapter presents interview questions related to specific domains

PSYCHIATRIC HISTORY

Number of admissions: (Severity of illness)

- *Is this the first time you are seeing a psychiatrist?*
- *Have you been admitted to hospital before?*
- *How many times have you been admitted to a mental health unit?*
- *When was your last admission?*
- *What has been your longest admission?*
- *What were you admitted for?*
- *Were your previous admissions for similar reasons?*

Treatment history: (What worked and What didn't, including side effects)

- *What diagnosis did the doctors give you?*
- *What medications were prescribed?*
- *Which were most helpful?*
- *How long did you take them for?*
- *Have you ever taken Lithium, Epilim (Valproate) or Tegretol (Carbamazepine)?*
- *Have you ever had ECT or shock therapy?*
- *Have you ever been on injections?*

Lithium, Valproate and Carbamazepine give clues for Bipolar Disorder

ECT indicates severity of depression

Depot medication indicates non-compliance

HISTORY OF SELF-HARM OR SUICIDE ATTEMPTS

- *Have you ever harmed yourself in the past?*
- *What did you do? (Go through details)*
- *What has been the most serious attempt?*
- *(ask how long patient stayed in emergency department/ICU, to establish severity)*



If several self-harm attempts exist, evaluation of personality is important

PSYCHOLOGICAL THERAPIES

- *Have you ever seen a counsellor or a psychologist?*
- *Did you find it helpful?*
- *How long did you see the psychologist for?*
- *What was the therapy called?*
- *What didn't you find helpful?*

- ⊗ Leaving a psychologist or saying that psychologist wasn't helpful may indicate engagement and transference issues.

MEDICAL HISTORY

- *Do you have any medical problems?*
- *What medications do you take?*
- *Do you/have you had any significant medical illnesses such as diabetes, hypertension (high blood pressure), seizures/fits or a significant head injury?*
- *In cases of intravenous drug use, ask about HIV and Hepatitis.*

- ⊗ Certain medications are associated with psychiatric disorders

- **Interferon (HepC)** → depression
 - **Antiepileptics** → mood disorders and psychosis
 - **Benzodiazapines** → mood disorders
 - **Beta-blockers** → cause depression
 - **Steroids** → mood disorders
 - **Antihypertensives & Diuretics** → mood disorders
 - **ACE-I & Diuretics (Thiazide)** → caution with Lithium
 - **NSAIDS & Lithium** → can increase levels of Lithium
 - **Warfarin** → caution with carbamazepine (enzyme inducer)
 - **SSRIs and Diabetes** → SSRIs can reduce blood sugar by 30%
 - **TCAs and Diabetes** → TCAs can increase blood sugar by 130%
 - **HIV (HAART)** → associated with general psychiatric disturbance
- Medications should be used with caution in cases of liver and renal disease.
 - **Head injury** is associated with psychosis, depression, anxiety, personality change and cognitive deficits.
 - **Epilepsy**
 - Temporal lobe epilepsy can mimic schizophrenia
 - Associated with mood disorders including anxiety
 - High rates of suicide
 - Thyroid dysfunction, Polycystic Ovarian disease

FAMILY HISTORY

- *Tell me about your family*
- *Is there anyone in your family that has a significant mental health problem/issue?*
- *Has anyone in the family, and that includes cousins, uncles, grandparents, had a nervous breakdown, or been diagnosed with depression, bipolar disorder, manic depression, schizophrenia?*
- *Has anyone in your family ever committed suicide?*
- *What about problems with drugs, alcohol or violence?*

“I would like to backtrack a bit and ask you about your childhood in order to understand you as a person/to give me a better understanding of the issue.”

“Childhood is an important part of our lives. Our past colours the present to a great extent. Can you tell me a bit about your childhood?”

DIRECT QUESTIONING TO LEAD TO FORMULATORY MODELS

Birth

- *Where were you born? (Migration hypothesis)*
- *Were there any problems during your birth that you know of? (Obstetric complications: neuro-developmental hypothesis)*

Development

- *Did you have any illnesses as a baby (Attachment difficulties, neuro-developmental hypothesis)*
- *Were there any problems as a baby in talking or walking? (neuro-developmental hypothesis)*
- *Did you experience any delays in learning to talk or walk? (neuro-developmental hypothesis)*

Relationship with parents and siblings

- *What sort of memories of childhood do you have?*
- *How many siblings did you have and where were you in the birth/pecking order?*
- *Tell me about your parents?*
- *How did you get along with your parents?*
- *What did your parents do for a living?*

Lifetime risk of disorder if first-degree relative has disorder

	Lifetime risk if first-degree relative has disorder (%)	Lifetime risk in general population (%)
Bipolar Disorder	25	1
Schizophrenia	19	1
Bulimia Nervosa	10	2
Panic Disorder	10	4
Alcoholism	7	14
Anorexia Nervosa	5	1
Major Depression	3	1
ADHD	50	?

Clarification for possibility of abuse

- *How was discipline handled at home?*
- *Did that ever get physical such that you ended up with bruises or being hit?*
- *How did that affect you? (**Attachment and unmet dependencies model**)*
- *Was there any violence at home? (**Unmet dependencies and attachment model**)*
 - Male: identification with aggressor
 - Female: parallels in later life, self-blame, cognitive model
- *When you were a child, did anyone ever do something sexual that made you feel uncomfortable? (**attachment and unmet dependencies model**)*
- *Tell me about your relationship with your mother and father (**Harsh and critical – psychodynamic and cognitive model**)*

School

- *How did you go at school?*
- *Did you have many friends? (**schizoid/schizotypal personality; peer-group relationships –social and cognitive models**)*
- *Were you bullied?*
- *Did you wag school/play truant?*

Conduct Disorder

- *Did you get into trouble at school?*
- *Did you have difficulty concentrating?*
- *Did you get into fights?*
- *Did you get suspended or were you expelled?*
- *Did you ever damage property?*
- *Have you ever been cruel towards animals?*

ADHD

- *How did you do academically at school?*
- *Did you find it difficult to sit down at one place for a long time? (**Hyperactivity**)*
- *Did you find it difficult organizing tasks at school? (**Inattention**)*
- *Did you often make careless mistakes? (**Inattention**)*
- *Did you receive any special education? (**Developmental delay/learning difficulties**)*

Education and work

- *What age did you leave school?*
- *What did you do after you left school?*
- *How many jobs have you had until now?*
- *When was the last time you worked?*
- *What has been your longest period of employment?*
- *What has been your longest period of unemployment?*
- *Were you ever sacked?*

If evidence of vocational deterioration is identified, consider possibility of schizophrenia, organic disorder (e.g. sub-cortical deficits), drug and alcohol misuse, and therefore perform a cognitive evaluation of frontal lobe.

Relationships (sexual history)

- *How many significant relationships have you had?*
- *What has been your longest relationship?*
- *Do you have children?*

- *How do you get along with him/her?*



One may identify the possibility of personality dysfunction here by asking

- *What sort of person are you in relationships?*
- *Do you find that your relationships get unusually emotionally intense? (**Borderline Personality Disorder**)*
- *Are you sensitive to rejection or constantly fearful of being abandoned in relationships (**Borderline Personality Disorder**)?*

SOCIAL HISTORY

- Most of it should be covered in demographics
 - Very important in old age patients
 - Ask about carer, who is main carer, who does the shopping, cooking?
 - What additional help does the individual receive?
 - Physical aids
 - Design of house – single storey or multistorey
 - Disability Pension
 - Will
 - Guardianship
 - Power of Attorney
- } Only relevant in management

FORENSIC HISTORY

- *Have you ever been in trouble with the police?*
- *Have you ever been charged or convicted?*
- *What were the charges?*
- *Have you ever been to jail? How long?*
- *Have you ever been violent towards others?*
- *Have you ever been arrested for drink driving, drug possession, damage to property, shoplifting?*

PERSONALITY ASSESSMENT

BORDERLINE PERSONALITY DISORDER

DSM-IV CRITERIA FOR BORDERLINE PERSONALITY DISORDER

- Mood instability due to marked reactivity of mood (e.g. intense dysphoria, irritability or anxiety, usually lasting a few hours and only rarely more than a few days)
- Frantic efforts to avoid real or imagined abandonment
- Pattern of unstable and intense interpersonal relationships characterised by alternating between extremes of idealisation and devaluation
- Identity disturbance, persistently unstable self-image or sense of self
- Recurrent suicidal behaviour, gestures or threats or self-mutilating behaviour
- Chronic feelings of emptiness
- Inappropriate, intense anger or difficulty controlling anger

- Transient, stress-related paranoid ideation or severe dissociative symptoms
- Impulsivity leading to recklessness in areas that are potentially damaging (drugs, sex, eating disorder, reckless driving)

QUESTIONS TO ESTABLISH DIAGNOSIS

Affective Instability

- *Do you often have mood swings? /Do you find that your moods can shift from being happy to being sad in a matter of minutes or hours?*

Identity disturbance

- *Are you generally a confident person or do you have difficulty knowing who (Use person's name) is?*

Emptiness

- *Do you often feel as though you are empty inside?*
- *As though life is meaningless?*

Unstable and emotionally intense relationships

- *Tell me a bit about your relationships*
- *What sort of person are you in relationships?*
- *Are you particularly sensitive to rejection?*
- *Are you often worried about being abandoned in relationships?*
- *What do you do?*
- *Does that make you clingy?*

Inappropriate, intense anger

- *Are you the sort of person that would let other people know that you are angry, or do you bottle it up?*
- *Do you have difficulty controlling your anger?*

Impulsivity (Impulse dyscontrol)

- *Would you call yourself an impulsive person? ... a person who does things on the spur of the moment without thinking about the consequences.*
- *Has it ever got you into trouble?*

Chronic self-harm/suicidal behaviour

- *Looking back, when something has really gone wrong in your life, like losing a job or getting rejected, have you often done something to hurt yourself, like cutting yourself or overdosing?*
- *How do you feel after you self-harm?*

Transient stress-related paranoid/dissociative symptoms

- *Do you ever have strange experiences like hearing voices when no one else is around?*



IMPORTANT POINTS

There is a high degree of co-morbidity associated with Borderline Personality Disorder

- Depression
- Anxiety
- Substance misuse
- Complex PTSD
- Eating disorders

Past history will usually show

- Early childhood maladjustment
- Attachment difficulties
- School difficulties

ANTISOCIAL PERSONALITY DISORDER

Pervasive pattern of disregard for and violation of the rights of others occurring since the age of fifteen years, indicated by

- Failure to conform to social norms
- Deceitfulness
- Impulsivity
- Irritability and aggressiveness
- Reckless disregard for the safety of others
- Consistent irresponsibility
- Lack of remorse

Evidence of Conduct Disorder before fifteen years is present.

Establishment of diagnosis is usually by pattern of dysfunction through adolescence and adulthood.

SCHIZOTYPAL PERSONALITY (DSM-IV CRITERIA)

1. ***Cognitive distortions***
 - Ideas of reference
 - Odd beliefs or magical thinking
 - Odd thinking and speech
 - Suspiciousness or paranoid ideation
2. ***Perceptual distortions***
 - Transient unusual perceptual experiences
3. ***Social and interpersonal functioning:***
 - Inappropriate or constricted affect
 - Odd behaviour

- Lack of close friends or confidants
- Excessive social anxiety does not diminish with familiarity and tends to be associated with paranoid fears.

SCHIZOID PERSONALITY (DSM-IV CRITERIA)

Detachment from social relationships

Restricted range of expression of emotion in interpersonal settings.

1. Choose solitary activities
 2. Takes pleasure in few, if any, activities
 3. Lacks close friends or confidant
 4. Has little interest in sex
 5. Shows emotional coldness, detachment
- *Are you a people person, or someone who prefers to be alone?*
 - *Can you name a few things that you enjoy doing?*

DEPENDENT PERSONALITY

- *Who makes the decisions in your life?*
- *Are you an independent person, or have you relied on others to make important decisions in your life?*

Increased risk of depression, especially in case of loss of decision maker.

OBSESSIVE COMPULSIVE PERSONALITY

- *Are you a perfectionist?*
- *Are you the sort of person who needs to do things in a particular way so that if you don't you feel distressed?*

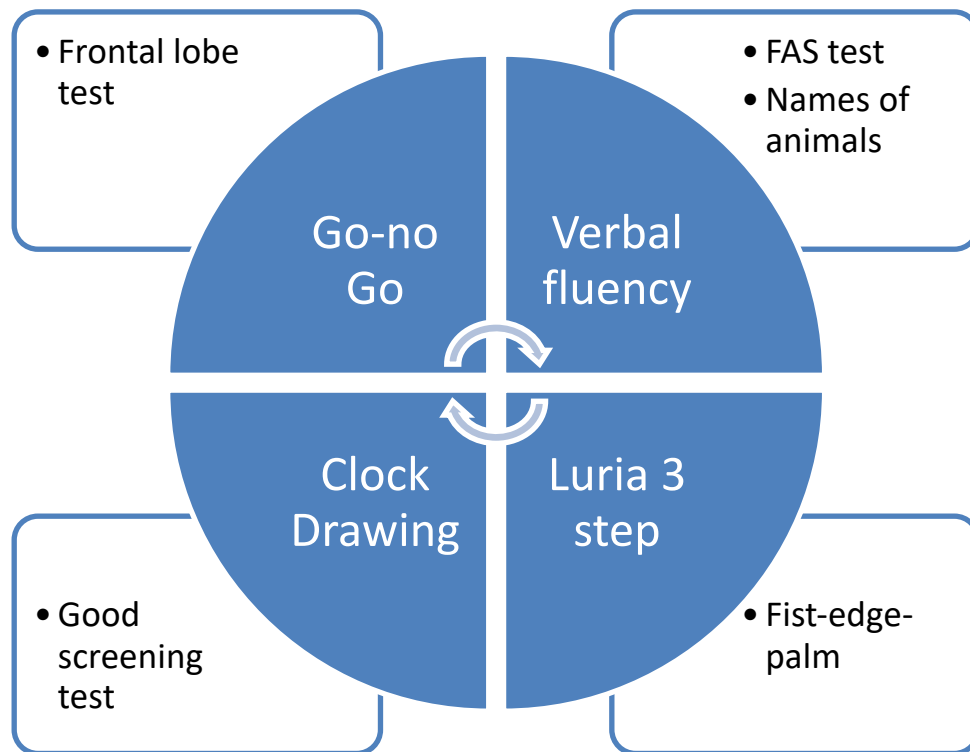
AVOIDANT PERSONALITY

- *Do you tend to avoid meeting people or getting close to people?*
- *Is it because you prefer to be alone or because you have been rejected before and you do not want it to happen again?*

NARCISSISTIC PERSONALITY

- *Do you often find yourself getting frustrated because other people do not meet your standards?*

COGNITIVE EXAMINATION



Other tests: MMSE, ACE-R, FAB, NUCOG etc.